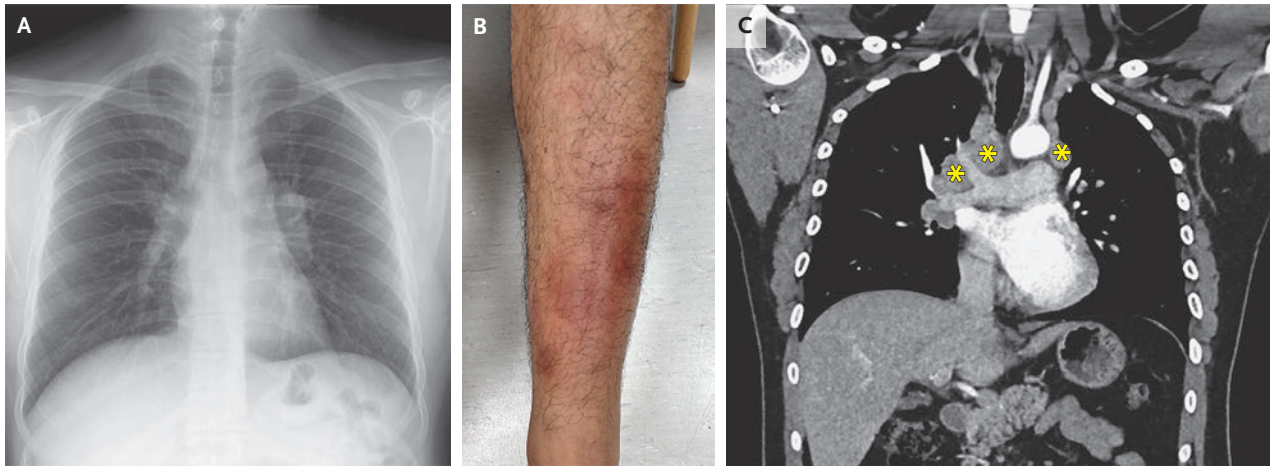


IMAGES IN CLINICAL MEDICINE

Löfgren's Syndrome



A 26-YEAR-OLD MAN WHO HAD EMIGRATED FROM NEPAL 7 YEARS EARLIER was referred to the pulmonology clinic with a 6-month history of remitting and relapsing ankle swelling, a 2-month history of cough and unintentional weight loss, and a 3-day history of leg rash. A chest radiograph obtained 6 weeks after the onset of the cough had shown hilar lymphadenopathy (Panel A). At the current presentation, physical examination was notable for erythematous, tender nodules and plaques on the anterior shins (Panel B). Swelling and tenderness were also noted in the ankles. Computed tomography of the chest showed enlarged mediastinal and hilar lymph nodes (Panel C, asterisks) and normal lung parenchyma. Owing to concern about possible tuberculosis, transbronchial biopsy of the right paratracheal and hilar lymph nodes was performed. Histopathological analysis showed noncaseating granulomas. Microbiologic studies for fungal and mycobacterial infections, including a nucleic acid amplification test for *Mycobacterium tuberculosis*, were negative. A diagnosis of Löfgren's syndrome — an acute form of sarcoidosis characterized by erythema nodosum, migratory polyarthrititis, and hilar lymphadenopathy — was made. A 6-week course of a tapering dose of an oral glucocorticoid was initiated, and the patient's condition improved quickly.

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